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Name: مديحه أنور محمود حميده National ID: 26906203200142	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Handwritten notes in Arabic script, including "DMS" and "مطهر فني" (Fayyaz Fanni).



شهادة شخصية
مادة حقوق الشخصية

مستجبه
المر محمد حبيب
القصر
م الواحات الداخلة - الوادي الجديد

٢ ٦٩ ٠ ٦ ٢٠ ٣٢ ٠ ٠ ١ ٤٣

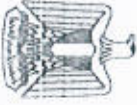
KZ4455175



٢٠٣٥/٠٢ ٢٦٩٠٦٣٠٣٢٠٠١٤٣
صاحبة ورشنة تجارة

متروجة
مسلمة
عزقة عمر محمد طاييه
البطاقة سارية حتى ٢٠٣٢/٠٣/٠٥

١٣





MDT FORM

/ / 20

G

Name	د. محمد الورد محمد	MRN	3826/2024
Age	55	ID	
Menopausal status	Post	Phone	
NRS SCORE		Marital status	
BMI		Family history	✓
CO morbidity	-✓	REASON OF MDT	

Clinical Data

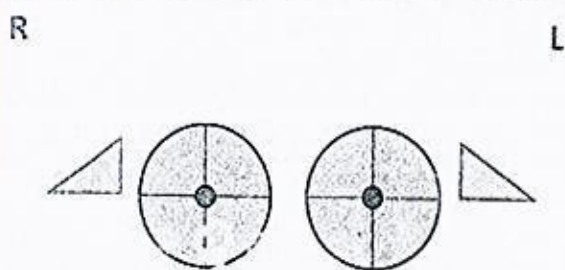
BX: IDC GTI

ER: 8/8 PR: 8/8 HER 2: +2 Ki67:

Sonomammography: Si SF. Amplified

Mets Workup:

- Multiple Bilateral Pulmonary deposits
- Multiple osseous lesions



TNM

Recommendations:

2017: TMRM no P. therapy - no Adjuvant HT (PT own history)
10/2024 -> MBC To Bone + local recurrence -> biopsy
GTI Triple Positive received 29 w/ Taxol - Herceptin
-> PR

MDT OUTCOME & PLAN

for herceptin + femara
+ Zometa

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name:	مديحة أنور محمود	
MR. :	5694	
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انها		
The patient has a history of left-sided breast cancer diagnosed in 2018, for which no adjuvant therapy was administered. The disease progressed to metastatic breast cancer involving bone, pleural effusion, lung, and a locally recurrent lesion. Pathology demonstrated estrogen-receptor positivity (8/8), progesterone-receptor positivity (7/8), and HER2-positive disease (HER3+). The patient was treated with taxol, trastuzumab (Herceptin), and letrozole (Femara) beginning in July 2025, with subsequent bone scan (BS) in January 2026, radiation therapy to the left axillary lymph nodes, and management of a lung collapse with pleural thickening. An echocardiogram on 12 January 2026 showed a left ventricular ejection fraction of 66 % while on trastuzumab 440 mg and Femara. The current plan includes continuation of trastuzumab 440 mg and Femara. The patient underwent ultrasound-guided core biopsy of the left breast recurrence and bilateral axillary lymph nodes, confirming disease recurrence (cT4). The treatment plan was continued with trastuzumab IV 440 mg and Femara for the new clinical status. For assessment, this was documented as the patient's 17th cycle of trastuzumab IV and the 5th total cycle of the same regimen as of the most recent evaluation. The patient declined a repeat echocardiogram; a future echocardiogram will be scheduled to monitor cardiac function during the 17th trastuzumab IV cycle and ongoing Femara therapy. The patient has therefore completed 17 cycles of trastuzumab IV and 5 total cycles of this regimen to date. On physical examination, there were palpable left axillary lymph nodes measuring 2 × 2 cm and left cervical lymph nodes measuring 2 × 2 cm, consistent with new clinical findings. A repeat bone scan was ordered for assessment of disease (cT8). The patient continues on trastuzumab 440 mg and Femara, with an estimated ejection fraction of 60 % as of 6 August 2026, pending a dental examination.		
تحريرا في ,		
	الوقت :	التاريخ : 19-09-2026
dina makram		الطبيب المعالج :
يعتمد ,		

patient Name : الأستاذة / منيرة نور محمود حبيب
 patient No. : 26903203200142
 No. : 3816/24
 : Female
 : 55 Y
 referring Doc :

Request No. : 13008787
 Request Date : 08/10/2024
 Verification Date : 16/10/2024
 Financial Category : كورنيت دار السلام - عيانت
 Department : Out Patient
 Registered by : Ali mostafa

Pathology Report

clinical Data: Left breast MRM with operative bed masses.
 Nature of Specimen: Tru-cut biopsy, collected at 7/10/2024.

gross Examination:
 three grayish white tissue cores up to 2 cm, totally submitted.

microscopic Examination:
 histological type: Invasive duct carcinoma.
 grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
 ductal carcinoma in situ: Negative.
 perineural invasion: Negative.
 angiolymphatic invasion: Negative.
 microcalcifications: Negative.

Diagnosis:
 Left breast MRM with operative bed masses, Tru-cut biopsy:
 Invasive duct carcinoma, Grade II.
 for further IHC (ER, PR, Her2, Ki67).

Signature
 Dr. Walaa Ghonaim.
 Dr. Toka Medhat.

Verified by:
 Dr. Toka Medhat

Printing Date : 31/10/2024 11:29:16

Page 1 of 1

Printing By : Mrs. Aya Mohamed

nt Name : الأستاذة / منيرة النور محمد

nt No. : 26906203200142

No. : 3816/24

: Female

: 55 Y

reing Doc. :

Request No. : 13006787

Request Date : 09/10/2024

Verification Date : 24/10/2024

Financial Category : دور / شهر - مائة

Department : Out Patient

IHC No. :

Registered by : Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

al Data: Left breast MRM with operative bed masses.

e of Specimen: Tru-cut biopsy.

lique:

ions were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana)

ained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and

toxylin as counter stain. The sections treated against:

clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

ts:

(Positive control showed appropriate reactivity):

Strong nuclear reaction in 98% of tumor cells (score 8/8).

Moderate nuclear reaction in 70% of tumor cells (score 7/8).

2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).

7: 60% nuclear reaction of tumor cells.

lusion:

reast MRM with operative bed masses, Tru-cut biopsy:

ive duct carcinoma, Grade II.

Positive (score 8/8).

Positive (score 7/8).

2: Equivocal (score 2+) For (SISH).

7: Positive in 60% of tumor cells.

ring system: "ER, PR"

Verified by :

DR.Nermeen Mohammed

nting Date : 31/10/2024 11:29:25

Page 1 of 2

Printed By : Mrs.Aya Mohamed

int Name: الأ.د.د. / منيحه النور محمود حميدة
int No.: 26906203200142
No.: 3816/24
Female
55 Y

Request No.: 13006/87
Request Date: 08/10/2024
Verification Date: 24/10/2024
Financial Category: وزارة الصحة - عين شمس
Department: Out Patient

Referring Doc.

Registered by: Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, between 1% and 10%; 2, between 11% and 33%; 3, between 34% and 56% and 4, between 57% and 100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

Reporting system: "Her2"

ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor

Signature

Dr. Walaa Ghonim.

Dr. Menna Nabil.

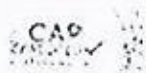
Verified by :

DR.Nermeen Mohammed

Printing Date : 31/10/2024 11:29:25

Page 2 of 2

Printed By : Mrs.Aya Mohamed





Name: Mrs. Madeha Anwar Mahmoud Mamceda

Date: 12/10/2024

ID:2224065877

Acc.:5039017

Referring Prof. Dr. Hanan Ezzat Shafiq AWRAM.

Tc-99m MDP whole body bone scan

Technique:

- Whole body images were acquired 180 minutes after the IV injection of ~15 mCi of the tracer.

Findings:

- Multiple focal spots of active tracer uptake are seen in skull bones, both shoulders, both humeri, sternum, left clavicle, multiple dorsal and lumbar spine, multiple bilateral ribs, sacrum, pelvic bones, both femora, right fibula.

Opinion:

Multiple metastatic osseous deposits, for follow up bone scan

Truly yours

Prof. Dr. Ahmed Kandeel, MD

Prof. of Nuclear Medicine, faculty of medicine, Cairo University

Printed by AR

Patient Name: Madiha Anwar Mahmoud Hmida	Date : 04/11/2024
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 55 Year(s)
Medical ID: 3826/2024	Pat. ID: 80380

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Left breast MRM showing retro-areolar irregular soft tissue mass measures 1x2.7cm inseparable from nipple with overlying skin retraction, other small related outer quadrants nodules, the lesion is seen inseparable from under pectorals muscle associated with skin thickening and enhancing cutaneous nodules measuring about 8x7 mm.
- Left para-sternal soft tissue lesion seen infiltrating the underlying pectoralis muscle measures 1.4x1.2cm
- Enlarged bilateral axillary level I lymph nodes largest seen on the left side measures 1.7x1cm, and to the right side measures 1.7x1.5cm.
- Right sided moderate pleural effusion is seen with subsegmental relaxation collapse and subtle mediastinal shift to the left side
- Right lung shows upper lung lobe peribronchial thickening with areas of ground glass opacities and consolidative patch is seen mounting to anterior mass formation almost replacing the right upper lung lobe with encasement and narrowing the right upper pulmonary artery.
- Multiple bilateral pulmonary nodule of variable densities, largest seen are seen at superior segment of left lower lung lobe measures 8x7mm
- Few small subcentemetric prevascular lymph nodes.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- Subtle pericardial effusion .
- Normal CT appearance of the thoracic aorta and the great vessels.
- Port A cath is seen.

II Abdomen & pelvis:

Patient Name: Madiha Anwar Mahmoud Hmida	Date : 04/11/2024
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 55 Year(s)
Medical ID: 3826/2024	Pat. ID: 80380

- The liver is enlarged in size with homogenous CT texture. No definite enhancing focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:**

- Wide spared axial and appendicular sclerotic lesions.
- No vertebral structural collapse or associated soft tissue component.
- L4/5 1st degree degenerative spondylolisthesis.

Opinion:

- Left breast malignant process with bilateral axillary lymph-nodes, for ultrasound correlation.
- Right sided pleural effusion with lower lung relaxation collapse.
- Right upper lung lobe consolidation mounting to mass formation for follow up or PET/CT correlation.
- Bilateral pulmonary nodules could be metastatic.
- Wide spread sclerotic osseous metastatic deposits.

Dr/Reem Elwaly

Dr. Reham Anwar, FEBR, MD